NCCP 00376 · VERSION 4

Exemestane · daily hormone therapy

Take exemestane once daily continuously, preferably after a meal. There is no planned week off. Confirm the prior-treatment and duration plan.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Exemestane: material risks

☐ Joint and muscle pain or stiffness

☐ Vaginal dryness and sexual symptoms

☐ Lipid / cardiovascular risk changes

☐ Hot flushes, sleep disturbance and fatigue

☐ Bone thinning and fractures

☐ Uncommon liver abnormalities or serious allergic reactions

The kidney and liver advice for exemestane differs from some other aromatase inhibitors: a routine dose reduction is not specified, but organ problems still need clinical review. Tell the pharmacist about all medicines and supplements because some can make exemestane less effective. Do not increase the dose yourself. Confirm menopause and any separate ovarian-suppression plan.

Individual benefit, alternatives and concerns

Agreed checks and next review

FBC, renal and liver profile, confirmation of menopause when uncertain, and bone / fracture assessment. Consider baseline vitamin D as specified by the product information. The NICE adjuvant baseline-DXA recommendation has an exception for patients already receiving adjuvant bisphosphonates. Review symptoms, adherence and bone health. Repeat bone density and lipids when indicated; assess other blood tests according to the clinical plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.